June 11, 2025

Dr. Thomas Keane
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C St SW
Floor 7
Washington, DC 20201

RE: Request for Information; Health Technology Ecosystem

Dear Dr. Keane,

The American Association of Nurse Practitioners (AANP), representing the more than 431,000 nurse practitioners (NPs) in the United States, appreciates the opportunity to provide comment on the Request for Information (RFI) for the heath technology ecosystem. We support the strategic goals of utilizing Health IT to lead infrastructure progress to cultivate the market, increasing beneficiary access to effective digital capabilities needed to make informed health decisions, and increasing data availability for all stakeholders contributing to health outcomes. We look forward to a continued partnership with ASTP to ensure that health information technology(IT) is deployed responsibly and empowers NPs to continue to provide high quality patient care.

As you know, NPs are advanced practice registered nurses who are prepared at the masters or doctoral level to provide primary, acute, chronic and specialty care to patients of all ages and backgrounds. Daily practice includes assessment; ordering, performing, supervising and interpreting diagnostic and laboratory tests; making diagnoses; initiating and managing treatment including prescribing medication and non-pharmacologic treatments; coordinating care; counseling; and educating patients and their families and communities. NPs hold prescriptive authority in all 50 states and the District of Columbia (D.C.) and perform more than one billion patient visits annually. Currently, twenty-seven states, the District of Columbia and two U.S. territories have adopted full practice authority (FPA), granting patients full and direct access to nurse practitioners.[1]

NPs practice in nearly every health care setting including hospitals, clinics, Veterans Health Administration and Indian Health Services facilities, emergency rooms, urgent care sites, private physician or NP practices (both managed and owned by NPs), skilled nursing facilities (SNFs) and nursing facilities (NFs), schools, colleges and universities, retail clinics, public health departments, nurse managed clinics, homeless clinics, and home health care settings.

NPs have a particularly large impact on primary care as according to the Medicare Payment Advisory Commission (MedPAC), APRNs and PAs comprise approximately one-third of our primary care workforce, and up to half in rural areas.[2] NPs are also "significantly more likely than primary care physicians to care for vulnerable populations. Nonwhites, women, American Indians, the poor and uninsured, people on Medicaid, those living in rural areas, Americans who qualify for Medicare because of a disability, and dual-eligibles are all more likely to receive primary care from NPs than from physicians."[3] NPs provide a substantial portion of health care in rural areas and areas of lower socioeconomic and health status. As such, they understand the barriers to care that face vulnerable

[1] https://www.aanp.org/advocacy/state/state-practice-environment.
[2] https://www.medpac.gov/wp-content/uploads/2022/06/Jun22_MedPAC_Report_to_Congress_SEC.pdf (see Chapter 2.)
[3] https://www.aei.org/research-products/report/nurse-practitioners-a-solution-to-americas-primary-care-crisis/

populations on a daily basis.[4],[5],[6]

With nearly one billion patient visits performed every year, NPs utilize health IT such as electronic health records (EHRs) daily. The continued integration and scaling of these technologies, combined with patient portals, and the continued growth of telehealth, are all challenges for clinicians as they focus on their most important mission: providing quality care to patients.

While our members have experienced the benefits of these technologies, they have also noted the growing challenges associated with them. We appreciate that the areas identified by ASTP focus on the most important aspects of health IT, which are empowering the patients and the providers who are the end users. As technology continues to be integrated into clinical practice, it is important that it is designed and implemented with the goals of reducing provider burden and equitably providing patients with greater access to care. Included below are our comments on the specific prompts included within this RFI.

C. Providers

1. Digital Health Apps

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

We support the ability of individuals to manage their health through technology, and safeguards which ensure effective utilization of these technologies. We would respectfully note that health IT is utilized in all clinician workflow, not just physician, and respectfully request that ASTP utilize "provider workflow" in this objective. As providers of whole person, patient centered care, NPs support empowering patients to manage their care. This RFI recognizes some of the challenges associated with patient utilization of health IT, including access to technology, broadband, privacy, and literacy. We support policies which empower patients in accessing and using their electronic health information (EHI) securely, privately, and without special effort. These guardrails are critical to ensuring that every patient can access their own health information in their preferred way. Promoting access to health IT resources is a critical component of any digital health strategy. As trusted sources of care, NPs will be important conduits of information to patients.

Therefore, promoting education, outreach and transparency on the use of artificial intelligence (AI) technologies is critical to patients and providers for the future. There is a need to inform patients and providers about the use of AI technologies in health care, and ensure transparency in its implementation to assess performance, quality, and integration into clinical practice and EHRs. As the landscape continues to grow in scale and complexity, providers will rely on ASTP to provide them with non-biased information.

It is essential that the utilization of health IT and other modern technologies in clinical workflows is implemented with a goal of supporting clinicians. It is important that health IT supports clinicians and does not increase burden. Providers must be included within the design and implementation of these technologies to ensure they are empowering providers to provide high-quality, safe, efficient and

[4] Davis, M. A., Anthopolos, R., Tootoo, J., Titler, M., Bynum, J. P. W., & Shipman, S. A. (2018). Supply of Healthcare Providers in Relation to County Socioeconomic and Health Status. Journal of General Internal Medicine, 4–6. https://doi.org/10.1007/s11606-017-4287-4.

[5] Xue, Y., Smith, J. A., & Spetz, J. (2019). Primary Care Nurse Practitioners and Physicians in Low-Income and Rural Areas, 2010-2016. Journal of the American Medical Association, 321(1), 102–105.

[6] Andrilla, C. H. A., Patterson, D. G., Moore, T. E., Coulthard, C., & Larson, E. H. (2018). Projected Contributions of Nurse Practitioners and Physicians Assistants to Buprenorphine Treatment Services for Opioid Use Disorder in Rural Areas. Medical Care Research and Review, Epub ahead. https://doi.org/10.1177/1077558718793070

evidence-based care. In order for providers to be empowered, it is critical that the federal government promote greater interoperability and secure health information sharing through nationally adopted standards, increase transparency and understanding of health data that goes into algorithm-based decision support tools, and advance standardization and interoperability of social determinants of health data. Each of these plays a critical role in ensuring that providers can access patient data, and patients are empowered to manage their care.

It is critical that health IT policies reduce regulatory and administrative burden. This is one of the biggest concerns identified by our members, that greater utilization of health IT has actually increased burden on providers instead of streamlining clinical workflow. It is critical that the plans simplify and streamline electronic documentation requirements. The broader usage of policies such as prior authorization, and requirements for specific notes, have led to a greater burden for providers. AANP has strongly supported the implementation of regulations governing the usage of prior authorization.

The federal government and ASTP will continue to play a critical role in ensuring responsible adoption of these technologies in a way that does not increase the burden on providers. Leveraging health IT to standardize data and processes related to electronic prior authorizations to allow for increased automation is essential to achieve these goals. However, we caution that automation of these processes must come with enhanced transparency and safeguards to ensure that patients are able to access the care they need.

We also support that the health care workforce is empowered to utilize health IT with confidence. Supporting health care providers through responsible use of health IT is critical to ensuring these tools are designed to empower providers. Improving experiences through the other objectives in this RFI is critical to the success of health IT.

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?

It is essential that regardless of the setting of care, patients and providers are empowered to access data. In the Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability (HTI-2) proposed rule, ASTP proposed to adopt a set of certification criteria to support data exchange between health care payers, providers, and patients. These proposed certification criteria would enable the exchange of data including clinical and coverage information, drug formulary information, and prior authorization information, between patients, providers, and payers as appropriate to each exchange. These proposed certification criteria are based on a series of recent policies finalized by the Centers for Medicare and Medicaid Services (CMS).

We support many of the provisions included within the CMS finalized rule to better support coordinated care for patients and providers.[7] Inclusion of further information for patients and ensuring that providers understand what is required of them through electronic prior authorization processes should reduce burden and remove barriers. It is important that the adopted standards are uniform across APIs to ensure that providers and staff are able to work with a streamlined interface and workflow, and information is uniform across these systems.

Many times providers are faced with their own EHR technology, combined with payer platforms and others, which can lead to burnout and confusion. While this proposed section is a voluntary certification, we agree with the agency that enabling the "exchange of information that promotes a more effective marketplace, increases competition, and provides benefits to patients, including: increased consumer

[7] 89 FR 8758

choice, improved outcomes in healthcare services, and more robust care coordination through improved availability and exchange of health care provider information."[8] We continue to support the efforts of CMS and ASTP to ensure that technology improves care for patients and providers while reducing burden.

2. Data Exchange

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

Align Federal Criteria

We support aligning criteria across federal agencies for one set of uniform regulations. For example, in the HTI-2 proposed rule, ASTP proposed to align ASTP's criteria with the criteria established by CMS in the Interoperability and Prior Authorization rule. In the CMS proposed rule, we strongly agreed with the agency that "with patient data scattered across these disconnected systems, it can be challenging for providers to get a clear picture of the patient's care history, and patients may forget or be unable to provide critical information to their provider.[9] This lack of comprehensive patient data can impede care coordination efforts and access to appropriate care."[10] Better access to comprehensive data will empower both patients and providers to be partners in a patient's plan of care. Therefore, we supported the proposal to add information about prior authorizations to the categories of data required to be made available to patients through the Patient Access API.

Real-Time Prescription Benefit Certification Criterion

In the HTI-2 proposed rule, ASTP proposed to establish a real-time prescription benefit (RTPB) certification criterion.[11] As noted by the agency, "the increasing costs of prescription drugs have long been a concern for patients, providers, and policymakers. Increased drug costs can have several negative consequences for patients, including limited access to healthcare, lower healthcare use, medication nonadherence, and financial stress, especially among underserved, uninsured and underinsured populations."[12] Further, we agree that "Real-time prescription benefit tools empower providers and their patients to compare the patient-specific cost of a drug to the cost of a suitable alternative, compare prescription costs at different pharmacy locations, view information about out-of-pocket costs, and learn whether a specific drug is subject to utilization management restrictions such as prior authorization, step therapy, or quantity limits."[13]

We appreciate that this proposed rule highlighted the impact patient cost, as well as insurer policies such as utilization management, step therapy, prior authorization, and quantity limits have on patients and providers. Often, it is the provider who is working with the patient through all these different complexities to ensure that they are receiving the care they need within the limitation of their plan and coverage. Not having this information available in real-time can cause communication issues between patients and providers, and lead to patients not receiving their needed medications. We agree that the "use

[8] 89 FR 63580
[9] 89 FR 8758
[10] Promoting Interoperability Performance Category: Traditional MIPS Requirements (cms.gov)
[11] Federal Register :: Health Data, Technology, and Interoperability: Patient Engagement, Information Sharing, and Public Health Interoperability
[12] 89 FR 63530
[13] Ibid

of RTBTs enables Medicare providers and enrollees to make cost-informed decisions about prescriptions, and a standardized approach will ensure that critical drug and drug price data is available to providers when they need it."[14]

Therefore, we agree that "use of these tools can allow the provider and patient to choose among clinically acceptable alternative medication treatments while weighing coverage and point-in-time costs. Access to this data within the electronic prescribing workflow may also help to reduce provider burden associated with coverage determination and prior authorization appeals. We believe widespread adoption of such tools, along with increased awareness of drug cost information among patients and providers will likely spur more robust evaluations over time."[15] However, as we have noted in previous comments to agencies, complicated prior authorization, prescription management and step therapy policies can be challenging and time consuming for providers.

A lack of uniformity across payers, insufficient response times, denials without justification, and complications with interoperability all lead to barriers which inhibit access to equitable care. We broadly support a real-time prescription benefit, which would help expedite some of these processes. However, we do not believe that merely sharing information on these policies is the answer, and broadly support needed reforms to the prior authorization processes. Establishing a "real-time prescription benefit" health IT certification criteria and including this certification criteria within the Base EHR definition, will help ensure uniformity across systems and hopefully reduce the burden for patients and providers.

However, we do not believe that ASTP establishing new requirements should lead to a technology "option" from EHR companies, despite it being a requirement from ASTP for health care providers. This could potentially shift the financial burden to health care providers, as companies could then offer this required technology as an "option" and require a further financial commitment.

Addressing Anti-Competitive Practices

We encourage coordination between CMS and ASTP to ensure consistency across standards to minimize burden on providers. It is important that these technologies empower providers, and do not impose unnecessary restrictions. Therefore, ASTP must ensure through rulemaking that protections are in place which prevent payers from engaging in anticompetitive practices against providers. This includes ensuring that payers do not engage in anti-competitive practices against patients who choose a nurse practitioner as their health care provider.

In December 2020, the *Consolidated Appropriations Act of 2021*, which included the *No Surprises Act*, was signed into law by President Trump. Section 108 of the *No Surprises Act* requires three agencies; The Department of Labor, Department of Health and Human Services, and the Department of Treasury, to implement Section 2706 of the *Public Health Service Act* by promulgating rules prohibiting anticompetitive practices by commercial insurers against health care providers by January 1, 2022.[16] The previous administration took no action to begin this rulemaking, and as of this writing, no rules have been proposed or issued. As NPs are the providers of choice for patients across the country, these anticompetitive practices limit patient choice, and prevent our members from effectively providing the care their communities and patients need.

[14] Ibid
[15] Ibid
[16] BILLS-116hr133enr.pdf

Therefore, it is important that the provisions within a digital health strategy comply with the intent of Section 2706 and prevent anticompetitive practices. Therefore, we respectfully request ASTP include language within any proposal which clarifies that payers cannot engage in anticompetitive digital health practices against NPs and other clinicians based on their licensure.

Conclusion

We appreciate the opportunity to provide input on this RFI. We look forward to a continued partnership with ASTP to ensure that Health IT is deployed responsibly, equitably, and empowers NPs to continue to provide high quality patient care. Should you have comments or questions, please direct them to MaryAnne Sapio, V.P. Federal Government Affairs, msapio@aanp.org, 703-740- 2529.

Sincerely,

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Chief Executive Officer
American Association of Nurse Practitioners